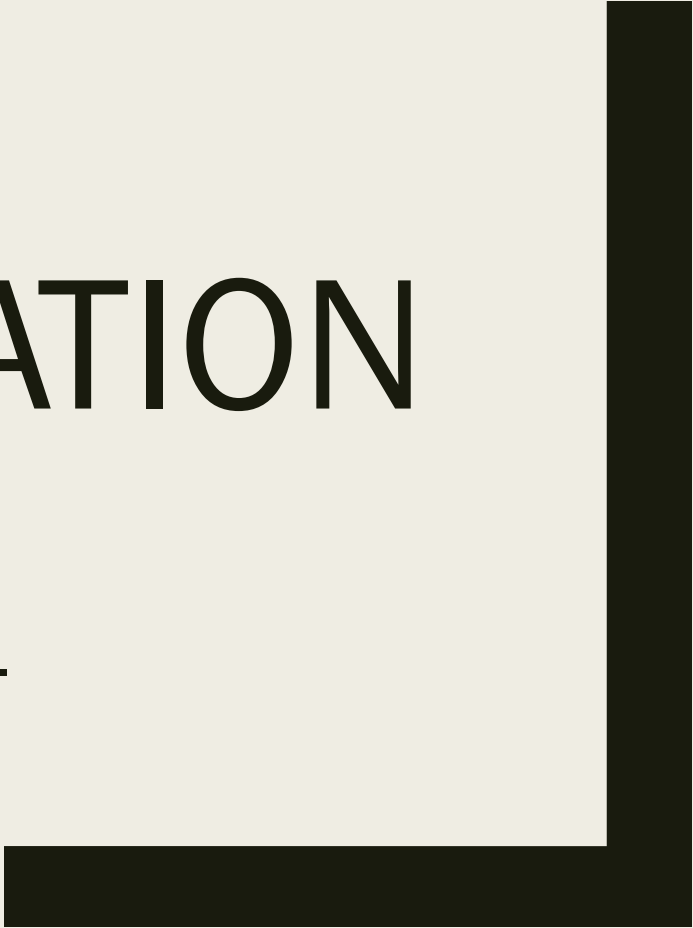




CASE PRESENTATION

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Particulars

- Name: Mrs. XYZ
- Age: 49 years
- Gender: Female
- Occupation: Traffic police
- Education: 10std
- Income: 20,000/month
- Address: Arali katte mane,Somarpet taluk,madikeri,571201
- Socio economic status: lower middle class
- Date of admission: 23rd april 2020
- Date of examinations : 25th april 2020
- History given by : patient

Chief complaint

Dilatation of veins in right leg since 3 years.

History of presenting illness.

Patient was apparently fine 3 years ago and then she noticed dilated veins on the back of her leg just below the knee joint. It was insidious in onset, there is prominence of vein in standing position and decreases in lying down position. Similar type of dilated veins are also seen on the medial side of right leg and thigh. It is associated with intermittent episodes of pain.

Pain is insidious in onset, present on the right leg, it is dragging type of pain, it aggravates while lying down and relieves after elevating limb, doing massage and on applying ice.

She even complains of itching on her right leg followed by color changes of skin from red to black. There is blackish discoloration of skin on her right ankle and foot.

No history of trauma,dialysis,abdominal pain, distention of abdomen , pregnancy, smoking, and Fever with calf pain.

No history of any wound, chest pain, breathlessness,hemoptysis, cough, long time immobilization of limbs.

Past history

- No similar complaints in the past
- Patient is known case of hypertension since 5 years and is under medication.
- No history tuberculosis, diabetes mellitus
- No history of surgery

Personal history

- Diet: vegetarian
- Appetite: normal
- Sleep: Adequate
- Bladder habits: regular
- Bowel habits: difficulty in passing stools, sometimes passage of blood in stools
- No addictive history

Menstrual history

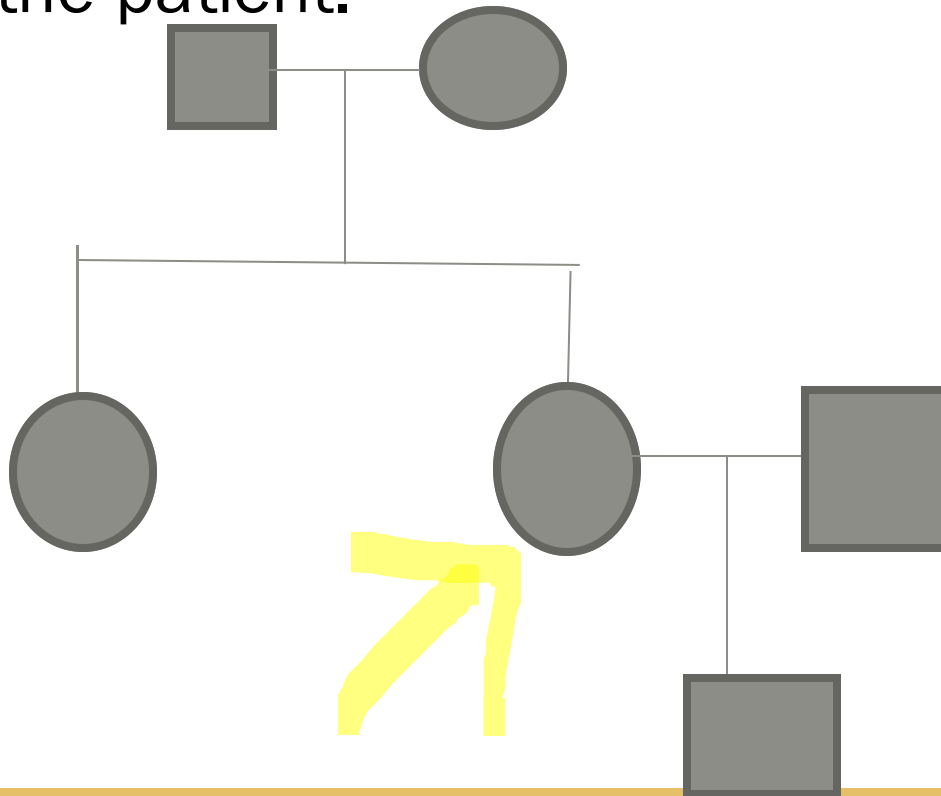
- Age of menarche: 14years
- Regular cycles: 28days +/- 3days
- Change of pads : 2-3/day
- No history of dysmenorrhea, passage of clots
- No history of consumption of oral contraceptive pills

Marital and obstetric history

- Married at age of 21years
- Number of children: 1
- Age at first pregnancy: 23years
- No history of abortion
- Normal vaginal delivery
- Breast fed the child till 1 year.

Family history

- Similiar complaints in the family that is **patient's** mother.
- History of hypertension in mother and father of the patient.



- Drug history: antihypertensive tablets
- Allergic history: no history of allergy to any substance.

Summary:

Here is the female patient, aged 49 years, traffic police by occupation presented with irregular swellings of vein on her right leg since 3 years.

She even complains of intermittent episodes of pain, which is dragging type, presence of itching and blackish discoloration of right foot and ankle.

She complains of difficulty in passing stools and some times passage of blood in stools.

At present she is not pregnant, no use of oral contraceptive pills, no wounds, fever, chest pain, breathlessness, hemoptysis.

From the history we can conclude that vascular system is involved

General physical examination

- Patient is comfortable at rest,conscious, co operative, moderately built with average nourishment and well oriented to time,place and person.
- BMI= 30Kg/m², height=150cm, weight=67.5kg
- No pallor
- No icterus
- No cyanosis
- No clubbbing
- No generalised lymphadenopathy
- Presence of bilateral pedal edema of pitting type.

■ Vitals:

1. Pulse rate: 72beats/min, regular rhythm,normal volume,normal charcter,no vessel wall thickening.
2. Blood pressure: 130/80 mm of Hg of left arm in sitting position
3. Respiratory rate: 17cycles/min, thoraco abdominal type.
4. Temperature: 37degree Celsius

Local examination

- Exposed both the limbs till umbilical region and patient is examined in standing position.

■ Inspection:

1. Attitude of the limb is normal
2. Localized dilatation of veins on right medial side of thigh, leg and posterior surface of knee joint.
3. These veins are prominent in standing position and decreases in supine position.
4. Blackish discoloration of right ankle and dorsal aspect of foot.
5. There no ulcer, or any scar, the skin is not stretched and shiny
6. There is loss of hair and increase brittleness of nails
7. Saphena varix is not present.
8. Other limb appears to be normal

Palpation

- Presence of local rise of temperature over the dilated veins
- No tenderness
- No thickening of veins

Tests.

- Brodie- trendelenberg test is positive at sapheno femoral junction.
- Tourniquets test is positive for communicating veins at sapheno femoral junction, mid thigh and below knee joint.
- Schwartz test is positive
- Pratts test, visible blow outs were seen at mid thigh and below knee perforators
- Morrissey's cough impulse test is positive
- Fegans test is positive and blow outs at mid thigh and below knee perforator.
- Perthes test is negative
- Modified perthes test is negative

■ Percussion

1. Shwartz sign is positive

■ Auscultation

1. No murmurs heard.

OTHER FINDINGS

- No enlargement of inguinal group of lymph nodes.
- All arterial pulsation felt.
- Other limb appears to be normal.

Other system examination.

■ Cardiovascular system:

1. Normal heart rate and regular rhythm
2. Heart sounds s1 and s2 heard.

■ Respiratory system

1. Normal vesicular breath sounds

■ Abdominal examination.

1. No mass per abdomen
2. No organomegaly

■ Examination of external genitalia, per vagina, per rectal was not done.

■ CNS examination.

1. No any neurological deficit present.

Provisional Diagnosis

- It is a case of varicose vein of right leg of clinical class C4a and primary etiology affecting great saphenous vein with sapheno femoral incompetence and perforator defect at mid thigh and below knee without any complications.
- CEAP classification C4aEpAsPr.

Investigation

- Duplex imaging
- Doppler ultrasonogram
- Radioactive fibrinogen test
- Phlebography

Other investigations

- Complete blood count
- Renal function test
- Liver function test
- ECG
- USG

■ Management

1. Compression
2. Endothermal ablation
 - Endo venous laser ablation
 - Radio frequency ablation
1. Ultra sound guided foam sclerotherapy
2. Open surgery- Sapheno femoral ligation and great saphenous stripping